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February 23, 2026

Dr. Thomas Keane

Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology

Attention: Request for Information: HHS Health Sector AI RFI

Mary E. Switzer Building Mail

Stop:7033A

330 C Street, S.W., Washington, D.C. 20201

RE: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13)

Dear Assistant Secretary Keane:

The Pharmaceutical Care Management Association (PCMA) appreciates the opportunity to provide comments on the Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RFI) published by the Department of Health and Human Services (HHS) Assistant Secretary for Technology Policy/Office of the National Coordinator of Health Information Technology (ASTP/ONC) in the Federal Register on December 23, 2025.¹

PCMA is the national association representing the country's pharmacy benefit managers (PBMs), which administer prescription drug plans (PDPs) and operate specialty pharmacies for more than 289 million Americans with health coverage through Fortune 500 companies, health insurers, labor unions, Medicare, Medicaid, the Federal Employees Health Benefits Program, and the Exchanges established by the Affordable Care Act. PBMs are hired by employers, unions, government programs, and others, to drive down prescription drug costs and administer prescription drug plans for more than 289 million Americans. Before getting into specifics on **RULE**, there are four things to know about PBMs:

- PBMs are the only part of the drug supply chain whose primary role is to lower prescription drug costs. On average, they save patients and families about \$1,154 per person each year.²
- PBMs are extremely effective at reducing prescription drug costs for employers and patients, which is why some industries that profit from high drug prices oppose them.
- For the enormous savings and value that PBMs provide, they operate on thin profit margins.

¹ 90 Fed. Reg. at 60108 (December 23, 2025).

² Visante. 2025. The Return on Investment (ROI) on PBM Services.

<https://www.pcmanet.org/wpcontent/uploads/2025/02/ROI-on-PBM-Services.pdf>

- Hiring a PBM is optional. Employers, unions, government programs, and others choose to use PBMs because they help lower drug costs and manage prescription benefits more efficiently. PBMs negotiate lower drug prices, process claims, and perform safety checks.

PBMs play a critical role at the intersection of clinical decision-making, benefit administration, and cost management. Our members have long-standing experience in using advanced analytics, automation, and decision-support tools to improve medication safety, ensure appropriate utilization, and lower costs for patients, plan sponsors, and taxpayers. Several of these tools are integrated directly into prescribers' workflows at the point of care, providing real-time information on coverage, cost-sharing, prior authorization requirements, and clinically appropriate alternatives to support informed treatment decisions. PCMA supports the responsible and ethical use of artificial intelligence (AI) to advance innovation and efficiency, and improve patient support. As AI-enabled tools become more prevalent in health care delivery and administration, it is essential that federal policy fosters responsible adoption while avoiding regulatory approaches that could inadvertently impede innovation, increase costs, or shift burdens onto patients and providers.

I. Regulatory Considerations

PCMA strongly supports a risk-based, use-case-specific approach to the regulation of AI in health care. AI applications vary significantly in function and risk, and regulatory oversight should reflect those differences. PBMs increasingly deploy AI-enabled tools to support coverage determinations that may involve a clinical component, including prior authorization and step therapy. These tools are designed to apply established clinical guidelines, benefit parameters, and plan sponsor requirements consistently and efficiently, often with human oversight and established appeal rights. When appropriately designed and governed, such tools can reduce administrative burden, improve consistency, and support timely access to medically appropriate care.

Federal policy should consider the level of risk involved in the deployment of AI solutions in crafting guardrails and, where possible, allow for oversight through existing federal and state frameworks, including Medicare Advantage and Part D requirements, ERISA fiduciary obligations, state utilization review laws, and civil rights protections. Layering new, technology-specific requirements on top of these frameworks risks creating duplication and uncertainty without corresponding benefits for patients. To ensure a consistent and trusted approach, PCMA supports the use of widely used and respected frameworks for responsible use of AI, such as the Organization for Economic Co-operation and Development (OECD) Principles on Artificial Intelligence, and the U.S. Department of Commerce's National Institute of Standards and Technology (NIST) Artificial Intelligence Risk Management Framework. We encourage HHS to work with NIST and other standards-development organizations to further develop and refine voluntary, consensus-based industry standards, as well as risk-based frameworks and best practices.

In order to build patient trust in the use of AI tools in clinical care, it is imperative that any regulatory framework requires that AI tools that involve clinical decision-making facilitate, but never substitute for, decision-making by clinicians, consistent with "human-in-the-loop" AI models. In addition, transparency about the use of AI tools and robust national privacy standards will help build the regulatory framework for AI to flourish.

Finally, as states have increasingly begun to regulate the use of AI, the multitude of varying state laws and requirements is becoming a barrier to the development and implementation of AI solutions. It is therefore essential that the appropriate regulation of AI in the health care sector be established at the federal level. This will provide clarity and consistency to ensure that the regulatory burden does not inhibit ongoing innovation to deliver better care and health outcomes.

II. Reimbursement Considerations: Aligning Incentives to Encourage Adoption

Although AI has demonstrated potential to reduce errors, improve efficiency, and lower costs, current reimbursement and payment structures often fail to recognize or reward its use. This misalignment presents a significant barrier to adoption, particularly in federal health programs where payment incentives strongly shape care delivery and administrative processes.

AI-enabled decision support can enhance medication therapy management, improve adherence, reduce inappropriate utilization, and streamline administrative workflows. These improvements benefit patients through better access and lower out-of-pocket costs, while also generating savings for plan sponsors and government programs. However, the entities that invest in AI infrastructure and governance often do not directly capture the value created under existing reimbursement models.

PCMA encourages HHS to consider how reimbursement policy can better support adoption of AI tools that demonstrably improve quality, reduce errors, and reduce total cost of care, particularly in areas where adoption has been slow or tentative because of investment, regulatory, or other hurdles. Potential approaches include incorporating the effective use of AI-enabled decision-support tools into value-based payment models, quality initiatives, and shared savings arrangements, and aligning AI use with performance benchmarks where providers have clear incentives to deploy tools that improve quality and efficiency. Additionally, HHS should ensure that reimbursement rules do not treat AI investments as purely administrative costs divorced from clinical value but should recognize that they also deliver meaningful value to patients and the health care system. Private payers should be allowed to use reimbursement models that incentivize the use of AI solutions that improve quality, efficiency, and more cost-effective care.

In Medicare and Medicaid, targeted demonstration projects could help evaluate how AI adoption by PBMs, plans, and providers affects outcomes, administrative efficiency, and program spending. Without clearer pathways for aligning incentives, adoption of AI is likely to remain uneven, limiting its ability to deliver system-wide benefits.

III. Research and Development: Supporting Innovation While Preserving Flexibility

PCMA supports continued federal engagement in AI research and development, particularly efforts that advance interoperability, data quality, and real-world validation. At the same time, R&D policy should be structured to complement private-sector innovation and avoid constraining model development or iteration.

PBMs and other stakeholders are actively investing in AI tools tailored to specific populations,

benefit designs, and clinical contexts. Federal efforts should prioritize access to high-quality data, promote public-private collaboration, and support research into bias mitigation and governance best practices, while allowing organizations flexibility to adapt tools as evidence and standards evolve.

Regulatory clarity is also essential to sustaining R&D investment. Clear signals that low-risk, non-autonomous AI applications will not be subject to disproportionate oversight will encourage continued innovation and responsible deployment.

Conclusion

AI and machine learning have the potential to improve clinical care, enhance administrative efficiency, and reduce costs across the health care system. PBMs are well-positioned to deploy these tools in ways that benefit patients, plan sponsors, and federal health programs. To fully realize this potential, federal policy should emphasize a risk-based regulatory framework and reimbursement models that align incentives with value, while relying on existing safeguards where appropriate.

PCMA appreciates the opportunity to provide input, and it looks forward to continued engagement with the Department as it advances policies to responsibly accelerate the adoption of AI in health care.

Thank you for your consideration of these recommendations.

Sincerely,

Tim Dube

Tim Dube, Senior Vice President, Policy, PCMA